Institute for Health Metrics and Evaluation
IHME DAH Database (2026)
User Guide

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Part 1: Introduction to the database

The IHME DAH Database enables comprehensive analysis of trends in international disbursements for health from sources of funding, channels of funding, for recipient countries/geographic regions, and health focus areas and program areas. In order to understand the framework used to track DAH, we recommend that users of the IHME DAH Database 2026 review the latest edition of the *Financing Global Health* report and its online methods appendix. Both resources can be found at https://www.healthdata.org/research-analysis/health-policy-planning/health-financing. Note that although this is our 2026 report, we only release estimates up to 2025 in this database.

Overview of variables: The main variable in this dataset is development assistance for health (DAH). The data are disaggregated at the year, source, channel, double counted transfers and recipient country level, so that these variables together are the unique identifiers.

> **Development Assistance for Health (DAH)** is the financial and in-kind resources transferred from major health development agencies to low-income and middle-income countries with the primary intent of improving or maintaining health. In this dataset, it is tracked from source to channel to recipient country, region, health focus area and program area. DAH is funded by the channel's corresponding income, or funds transferred from a source to the channel. Disbursements to specific health focus areas can include transfers between two channels, which can be captured in data from both channels. Duplicate transfers are removed before calculating DAH for each health focus areas, recipient region, etc. The dataset contains two elimination variables which identify double counting observations. These duplicate transfers are described on Page 3.
>
> **Source** distinguishes where funds originate from, such as private versus public funds from a particular donor country.
>
> **Channels** are the agencies through which funds are disbursed. (See Part 3 of this guide for detailed information on the definitions and admissible values of these variables).
>
> **Health Focus Areas (HFAs)** include Reproductive and Maternal Health (RMH), Neonatal and Child Health (NCH), HIV/AIDs, Malaria, Tuberculosis, Other Infectious Diseases (OID), Non-communicable Diseases (NCD), Sector-wide Approaches/Health System Strengthening (SWAP/HSS).
>
> **Program Areas (PAs)** are subcategories of HFAs detailed in Part 3.

All dollar values are reported in thousands of constant 2023 US dollars. Dashes indicate nonzero values under $500 that, when rounded to the thousands place, would be rounded to zero. Negative values in the dataset result from eliminations of double counted transfers between certain channels, and redistribution of DAH between countries for countries and years that were backcasted. Data was

backcasted for countries that divided into independent nations, such as Sudan and South Sudan in 2011, and Indonesia and Timor-Leste in 1999. Due to data limitations, regional- and country-level recipient data are not available after 2023.

Transfers between two channels, both of which are tracked in our database, must be removed before DAH can be summed. The code for removing these transfers can be found in Part 2 of this user's guide. These transfers include the following:

- **Funds flowing from the Gates Foundation (GATES), US foundations, GAVI, and the Global Fund to fight AIDS, Tuberculosis, and Malaria to other channels that we track, including NGOs**. These funds are in essence reported in the initial (first receipt) channel's data and the final (last receipt) channel's data. We track the funds at the final channel of disbursement, so the funds reported by the initial channel of receipt are dropped.
- **Funds flowing between UN agencies**. These include transfers between the Joint United Nations Programme on HIV/AIDS (UNAIDS), the United Nations Population Fund (UNFPA), the United Nations Children's Fund (UNICEF), the Pan American Health Organization (PAHO), and the World Health Organization (WHO). Funds reported by a UN agency channel as being sourced from another UN agency are dropped from the source agency. These funds are then captured in the outflow of the UN agency that received the funds. As Unitaid provides grantee information for the outflow of their funds, transfers to other agencies we track independently are dropped directly from the Unitaid analysis.
- **Funds transferred from bilateral agencies to NGOs tracked through 990 forms.** These transfers are reported by both bilateral agencies and NGOs. For DAH by channel estimates, the transfers reported by NGOs in the 990 forms are subtracted from each bilateral channel's DAH, and NGOs are kept as the channel. Since funds are reported by NGOs as being from US or non-US public sources, US public funds are subtracted from the US bilateral channel's DAH, and non-US public funds are subtracted from each non-US bilateral channel's DAH based on each channel's share of total DAH. For DAH by source estimates, the transfers reported by NGOs already tracked through the 990 forms are dropped and the transfers reported by bilateral agencies are kept.
- **Funds transferred from bilateral agencies to other channels that we track.** The OECD CRS dataset, through which we track funding contributed by bilateral agencies, also includes bilateral funding channeled through numerous other agencies including development banks, UN agencies, and public-private-partnerships. Since we track funding using data from those channels, we eliminate transfers from bilateral sources through these channels in order to avoid double counting. We identify double counted channel data using channel information in the CRS dataset, and drop funding disbursed through these channels.

Part 2: Using the database

The code below uses the IHME DAH Database 2026 to calculate DAH by channel, source, and health focus area. The code (1) removes transfers between channels that are captured more than once in the database, including transfers from BMGF to other channels, from one UN agency to another, and from GAVI and GFATM to other channels; (2) aggregates DAH by channel, source, or health focus area; (3) creates stacked bar graphs of disaggregated DAH over time. This example is written in R and depends on the data.table and ggplot2 packages. An example written in Stata is given further below.

```
# dah_database_example.R
#
# IHME Development Assistance for Health (DAH) Database Examples
#    FGH 2026
#
# 0. Loading and preparing the database
# 1. DAH by channel
# 2. DAH by source
# 3. DAH by health focus area
#
library(ggplot2) ## for creating plots
library(data.table) ## for fast data reading and manipulation


# define the path to the IHME DAH database
DATA_PATH <- "PUT THE PATH TO WHERE YOU STORED THE IHME DAH DATABASE"
DEFLATE_YR <- "23" ## base year for the DAH values in the database


#
# 0. Load the IHME DAH database ==============================================
#

# Loads data as a data.table (an enhanced data.frame) and removes double counting
dah_db <- data.table::fread(DATA_PATH)
dah_db <- dah_db[elim_ch == 0]



# The "*dah_yy" columns contain total DAH estimates and DAH broken out by health
# focus area and program area. This will be useful below.
dah_yr <- paste0("dah_", DEFLATE_YR)
```

```
#
# Convert DAH columns from string type to numeric type
# (the DAH database uses "-" to represent values that are less than $500 and thus
# would be rounded to 0 after the DAH estimates are rounded to thousands)
#
dah_cols <- grep(dah_yr, names(dah_db), value = TRUE)
dah_db[, (dah_cols) := lapply(.SD, function(col) {
  as.numeric(gsub("-", "", col))
}), .SDcols = dah_cols]



#
# 1. DAH by channel ============================================================
#

# Aggregate by channel-year
bychan <- dah_db[, .(dah_defl = sum(get(dah_yr), na.rm = TRUE)),
            by = .(year, channel)]

bychan[, dah_defl := dah_defl / 1e6] ## convert from thousands to billions USD

#
# Create channel groupings for better visualization - not necessary but makes
# for a cleaner plot
#
bychan[, channel_group := data.table::fcase(
  # other bilateral aid
  channel %in% c('BIL_ARE', 'BIL_AUT', 'BIL_BEL', 'BIL_CHE',
            'BIL_DNK', 'EC', 'BIL_ESP', 'BIL_FIN',
            'BIL_IRL', 'BIL_ITA', 'BIL_KOR', 'BIL_NLD',
            'BIL_NOR', 'BIL_NZL', 'BIL_SWE', 'BIL_GRC',
            'BIL_LUX', 'BIL_PRT', 'EEA'),
  "BIL_OTHER",
  # regional development banks
  channel %in% c('IDB', 'AsDB', 'AfDB'),
  "REG_DEV_BANKS",
  # other un agencies
```

```
    channel %in% c('UNICEF', 'UNFPA', 'UNAIDS', 'UNITAID', 'PAHO'),
    "UN_OTHER",
    # world bank
    channel %in% c("WB_IDA", "WB_IBRD"),
    "WORLD_BANK",
    # default - use channel
    rep_len(TRUE, .N), channel
)]

#
# Aggregate by group & create simple stacked bar plot ofDAH by channel over time
#
minyr <- min(bychan$year)
maxyr <- max(bychan$year)
agg <- bychan[, .(dah_defl = sum(dah_defl)), by = .(year, channel_group)]

ggplot(agg, aes(x = year, y = dah_defl, fill = channel_group)) +
  geom_bar(stat = "identity", position = "stack") +
  scale_x_continuous(breaks = seq(minyr, maxyr, by = 1)) +
  labs(title = paste("DAH by Channel,", minyr, "-", maxyr),
       x = "",
       y = paste0("DAH (Billions of 20", DEFLATE_YR," USD)"),
       fill = "Channel") +
  theme_bw() +
  theme(axis.text.x = element_text(angle = 45, hjust = 1))




#
# 2. DAH by source ============================================================
#
# To create a DAH by source of funding plot, we can follow the same
# process but instead of grouping by channel, group by "source".
#
# Here is a quick example:
bysrc <- dah_db[, .(dah_defl = sum(get(dah_yr), na.rm = TRUE)),
            by = .(year, source)]

ggplot(bysrc, aes(x = year, y = dah_defl, fill = source)) +
```

```
  geom_bar(stat = "identity", position = "stack") +
  scale_x_continuous(breaks = seq(minyr, maxyr, by = 1)) +
  labs(title = paste("DAH by Source of Funding,", minyr, "-", maxyr),
     x = "",
     y = paste0("DAH (Billions of 20", DEFLATE_YR," USD)"),
     fill = "Source") +
  theme_bw() +
  theme(axis.text.x = element_text(angle = 45, hjust = 1))




#
# 3. DAH by health focus area ================================================
#
# The data set contains both health focus areas (HFAs) and program areas (PAs),
# where the PAs are sub-categories of the HFAs. So be careful not to aggregate #
HFAs and PAs together.
# Here we will just focus on the broader HFAs:
hfas <- c("rmh", "nch", "ncd", "hiv", "mal", "tb", "oid", "swap_hss_total",
      "unalloc", "other")

byhfa <- dah_db[, lapply(.SD, sum, na.rm = TRUE),
          .SDcols = paste0(hfas, "_", dah_yr), ## e.g. "rmh_dah_23"
          by = .(year)]

#
# since the HFAs are columns, we can melt the data to make plotting easier
#
byhfa <- data.table::melt(byhfa,
                  id.vars = "year",
                  value.name = "dah_defl",
                  variable.name = "hfa")

# cleanup the resulting columns
byhfa[, dah_defl := dah_defl / 1e6] ## convert from million to billion USD
byhfa[, hfa := gsub(paste0("_", dah_yr), "", hfa)]

# Simple stacked bar plot of DAH by HFA over time
ggplot(byhfa, aes(x = year, y = dah_defl, fill = hfa)) +
```

```
    geom_bar(stat = "identity", position = "stack") +
    scale_x_continuous(breaks = seq(minyr, maxyr, by = 1)) +
    labs(title = paste("DAH by Health Focus Area,", minyr, "-", maxyr),
       x = "",
       y = paste0("DAH (Billions of 20", DEFLATE_YR," USD)"),
       fill = "Health Focus Area") +
    theme_bw() +
    theme(axis.text.x = element_text(angle = 45, hjust = 1))

# end
```

Part 3: Variable descriptions and values

Variable Name	Description	Admissible Value
year	Disbursement year	1990 – 2025
source	Denotes source	See Appendix A
channel	Channel of funding	See Appendix B
recipient_isocode **recipient_country**	Recipient country's ISO 3-digit code and country name (form used by IHME)	See Appendix A
gbd_location_id	Recipient country's Global Burden of Disease location ID	--
wb_regioncode	Recipient country's World Bank Region Code	See Appendix C
wb_location_id	Recipient country's World Bank Region ID	See Appendix C
gbd_region	Recipient country's Global Burden of Disease region	See Appendix D
gbd_region_id	Recipient country's Global Burden of Disease region ID	--
gbd_superregion	Recipient country's Global Burden of Disease super-region	See Appendix E
gbd_superregion_id	Recipient country's Global Burden of Disease super-region ID	--
elim_ch	Binary indicator to tag transfers between channels that are captured elsewhere in the database	1= drop to avoid double counting, 0 = Do not drop
prelim_est	Binary indicator to tag estimates based on preliminary data that may change in future editions (1 = Preliminary Estimate, 0 = Final estimate)	1= Preliminary Estimate, 0= Final estimate
dah_23	Total funds for health disbursed from source to channel to recipient country	
rmh_dah_23 **rmh_fp_dah_23** **rmh_mh_dah_23** **rmh_hss_other_dah_23** **rmh_hss_hrh_dah_23** **rmh_other_dah_23**	Funds for health disbursed from source to channel to recipient country for reproductive and maternal health, disaggregated by family planning, other health system strengthening, human resources, other maternal health, and other	--

Variable Name	Description	Admissible Value
nch_dah_23 **nch_cnn_dah_23** **nch_cnv_dah_23** **nch_hss_hrh_dah_23** **nch_other_dah_23**	Funds for health disbursed from source to channel to recipient country for newborn and child health, disaggregated by nutrition, vaccines, other health system strengthening, human resources, and other	--
hiv_dah_23 **hiv_treat_dah_23** **hiv_prev_dah_23** **hiv_pmtct_dah_23** **hiv_other_dah_23** **hiv_ct_dah_23** **hiv_ovc_dah_23** **hiv_care_dah_23** **hiv_hss_other_dah_23** **hiv_hss_hrh_dah_23** **hiv_amr_dah_23**	Funds for health disbursed from source to channel to recipient country for HIV/AIDS, disaggregated by care and support, counseling & testing, other health system strengthening, human resources, treatment, orphans & vulnerable children, prevention of mother to child transmission, prevention, drug resistance, and other.	--
mal_dah_23 **mal_diag_dah_23** **mal_hss_other_dah_23** **mal_hss_hrh_dah_23** **mal_con_nets_dah_23** **mal_con_irs_dah_23** **mal_con_oth_dah_23** **mal_treat_dah_23** **mal_comm_con_dah_23** **mal_other_dah_23** **mal_amr_dah_23**	Funds for health disbursed from source to channel to recipient country for malaria, disaggregated by community outreach, bednets, indoor spraying, other control, diagnosis, other health system strengthening, human resources, treatment, drug resistance, and other	--

tb_dah_23 **tb_other_dah_23** **tb_treat_dah_23** **tb_diag_dah_23** **tb_hss_other_dah_23** **tb_hss_hrh_dah_23** **tb_amr_dah_23**	Funds for health disbursed from source to channel to recipient country for tuberculosis, disaggregated by diagnosis, other health system strengthening, human resources, treatment, drug resistance, and other	--
oid_dah_23 **oid_hss_other_dah_23** **oid_hss_hrh_dah_23** **oid_ebz_dah_23** **oid_zika_dah_23** **oid_other_dah_23** **oid_amr_dah_23**	Funds for health disbursed from source to channel to recipient country for other infectious diseases, disaggregated by other health system strengthening, human resources, Ebola, Zika, antimicrobial resistance, and other	--
ncd_dah_23 **ncd_hss_other_dah_23** **ncd_hss_hrh_dah_23** **ncd_mental_dah_23** **ncd_other_dah_23** **ncd_tobac_dah_23**	Funds for health disbursed from source to channel to recipient country for non-communicable diseases, disaggregated by mental health, other health system strengthening, human resources, tobacco initiatives, and other	--
swap_hss_total_dah_23 **swap_hss_other_dah_23** **swap_hss_hrh_dah_23** **swap_hss_pp_dah_23**	Funds for health disbursed from source to channel to recipient country for health systems strengthening and sector-wide approaches, disaggregated by human resources, pandemic preparedness, and other	--
other_dah_23	Funds for health distributed from source to channel to recipient country for which we have health focus area information but which is not identified as being allocated to any of the other health focus areas listed	--
unalloc_dah_23	Funds for health disbursed from source to channel to recipient country for which we have no health focus area information	--

Appendix A: Source

Gates Foundation

Corporate Donations	Private sector in-kind contributions to NGOs
Debt Repayments	Debt repayments (World Bank)
Other OECD DAC Countries	Public sector funds from other OECD DAC countries including Czech Republic, Hungary, Iceland, Poland, Slovakia, Slovenia
Non_OECD_DAC_countries	Public sector funds from non-OECD DAC countries explicitly tracked (national treasuries)
Non_OECD_non_DAC_countries	Public sector funds from smaller donor countries not explicitly tracked elsewhere (national treasuries)
Other	Interest, transfer of funds, refunds, miscellaneous income earned by channel
Private_other	Private sector financial contributions (includes corporations, foundations, individuals, etc.)
Unallocable	Unspecified donor sector
Australia Austria Belgium Canada China Denmark Finland France Germany Greece Ireland Italy Japan Korea Luxembourg Netherlands New Zealand Norway Portugal Spain	Public sector funds (national treasuries)

Sweden
Switzerland
United Kingdom
United States

Appendix B: Channels of funding

Bilateral agencies:

BIL_ARE	United Arab Emirates
BIL_AUS	Australia
BIL_AUT	Austria
BIL_BEL	Belgium
BIL_CAN	Canada
BIL_CHE	Switzerland
BIL_CHN	China
BIL_DEU	Germany
BIL_DNK	Denmark
BIL_ESP	Spain
BIL_EST	Estonia
BIL_FIN	Finland
BIL_FRA	France
BIL_GBR	United Kingdom
BIL_GRC	Greece
BIL_IRL	Ireland
BIL_ITA	Italy
BIL_JPN	Japan
BIL_KOR	Korea
BIL_LTU	Lithuania
BIL_LUX	Luxembourg
BIL_NLD	Netherlands
BIL_NOR	Norway
BIL_NZL	New Zealand
BIL_PRT	Portugal
BIL_SWE	Sweden
BIL_USA	United States

Multilateral agencies:

EC	European Commission
EEA	European Economic Area

Public-private partnerships:

GAVI	Gavi, the Vaccine Alliance
GFATM	Global Fund to Fight AIDS, Tuberculosis, and Malaria
CEPI	Coalition for Epidemic Preparedness Innovations

Development banks:

AfDB	African Development Bank
AsDB	Asian Development Bank

IDB	Inter-American Development Bank
WB_IBRD	World Bank, International Bank for Reconstruction and Development
WB_IDA	World Bank, International Development Association

NGOs and foundations:

GATES	Gates Foundation
INTLNGO	International NGOs
NGO	US NGOs
US_FOUND	US Foundations

UN agencies:

PAHO	Pan American Health Organization
UNAIDS	Joint United Nations Programme on HIV/AIDS
UNFPA	United Nations Population Fund
UNICEF	United Nations Children's Fund
UNITAID	Unitaid
WHO	World Health Organization

Appendix C: World Bank Region Codes

Region code	Region name
EAP	East Asia and Pacific
ECA	Europe and Central Asia
LAC	Latin America and Caribbean
MNA	North Africa and Middle East
NA	Unallocable/Unspecified
SAS	South Asia
SSA	Sub-Saharan Africa
WLD	Global

Appendix D: Global Burden of Disease Regions

High-income Asia Pacific
Central Asia
East Asia
South Asia
Southeast Asia
Caribbean
Central Europe
Eastern Europe
Western Europe
Global
Andean Latin America
Central Latin America
Southern Latin America
Tropical Latin America
North Africa and Middle East
Oceania
Central Sub-Saharan Africa
Eastern Sub-Saharan Africa
Southern Sub-Saharan Africa
Western Sub-Saharan Africa
Unallocated/Unspecified

Appendix E: Global Burden of Disease Super-regions

Seven regions which group sub-regions based on cause of death patterns.

Central Eastern Europe Europe, and Central Asia High-income
GBD High-income
Latin America and Caribbean
North Africa and Middle East
South Asia
Southeast Asia, East Asia, and Oceania
Sub-Saharan Africa
Unallocated/Unspecified
Global